

RESEARCH PAPER 2

EMERGENCY MEDICAL SERVICE (EMS) IN INDIA: A CONCEPT PAPER

- National Health Systems Resource Centre

Abstract:

Emergency Medical Service (EMS) is provided by a variety of individuals, using a variety of methods. To some extent, these are determined by country and locale, with each individual country having its own 'approach' to how EMS should be provided, and by whom. In some parts of Europe, for example, legislation insists that efforts at providing Advanced Life Support (ALS) services must be physician-led, while others permit some elements of that skill set to specially trained nurses, but have no paramedics. Elsewhere, as in North America, the UK and Australia, ALS services are performed by paramedics, but rarely with the type of direct "hands-on" physician leadership seen in Europe. The goal of emergency medical services is to either provide treatment to those in need of urgent medical care, with the goal of satisfactorily treating the malady, or arranging for timely removal of the patient to the next point of definitive care. This is most likely a Casualty at a hospital or another place where physicians are available. The term Emergency Medical Service (EMS) evolved to reflect a change from a simple transportation system (ambulance service) to a system in which actual medical care occurred in addition to transportation. In some developing regions, the term is not used, or may be used inaccurately, since the service in question does not provide treatment to the patients, but only the provision of transport to the point of care. In India, EMS is a relatively new concept, where the most dominant model is the EMRI services. As of December 2009, more than 2,600 ambulances are operating under EMRI across around 10 states in India. Some other states like Bihar, Kerala, Himachal Pradesh and Delhi, have adopted EMRI-like model, but have some other agencies operating the EMS in those states. States like Madhya Pradesh and West Bengal had opted for basic transportation services (without stabilization care) in the PPP mode through multiple agencies (mostly NGOs) contracted at district/block level. The central government support to the above mentioned schemes is mainly in the form of capital expenditure (capex) support. Operating expenditure (opex) is borne by the states, with the central support being progressively reduced from 60% of opex to begin with, to zero by the 3rd year of operations. For the year 2010-11, total of Rs. 227.10 crores have been sanctioned under NRHM for 11 states (including 4 EAG states and 2 North-Eastern states) – for Emergency Referral Transportation System (ERTS), mostly through EMRI in PPP mode.